

MEXOMIDE TABLET

MAL19910164AZ

DESCRIPTION:

TABLET

Colour : White

Shape : Round, Flat and Scored

CONTENT:

Each Tablet Contains:

Metoclopramide Hydrochloride..... 10 mg

PHARMACODYNAMICS:

Metoclopramide increases gastric motility and accelerates gastric emptying. This action is antagonised by Atropine. It also has an anti-emetic action that is due partly to its local action on the gastrointestinal tract and partly to a direct action on the CNS.

PHARMACOKINETICS:

Metoclopramide is well absorbed after oral administration in animals. In rabbits, plasma concentrations of about 90 mcg/ml are attained after 30 to 120 min after oral dose of 200 mcg/kg. In rats and dogs, plasma half-life are 60 to 90 min. In rabbits about 2 hours. In mice, Metoclopramide is widely distributed throughout the body with highest concentrations appearing in the liver, intestinal mucosa and biliary tract, and lesser concentration appearing in the heart, adrenal glands, thymus, fat, bone marrow and CNS. In animals, Metoclopramide appears to be subject to enterohepatic circulation; volume of distribution in rats, rabbits, and dogs greater than 1.1 L/kg body-weight; protein binding, moderately bound to plasma proteins in man. In rats and dogs, metabolised by O-demethylation, N-de-ethylation, and amide hydrolysis then conjugated with glucuronic acid and sulphate; in rabbits, possibly the formation of N4-acetyl derivatives; in animals there appears to be significant first pass metabolism in the liver. After an oral dose of 10 mg, 50% is excreted unchanged in 8 hours; in rats and dogs about 50% is excreted mainly as N-dealkylated or O-demethylated metabolites.

INDICATION:

Adult population

MEXOMIDE TABLET is indicated in adults for:

- Prevention of delayed chemotherapy induced nausea and vomiting (CINV).
- Prevention of radiotherapy induced nausea and vomiting (RINV).
- Symptomatic treatment of nausea and vomiting, including acute migraine induced nausea and vomiting.

Paediatric population

MEXOMIDE TABLET is indicated in children (aged 9-18 years) for:

- Prevention of delayed chemotherapy induced nausea and vomiting (CINV) as a second line option.

RECOMMENDED DOSE:

Adults:

The recommended single dose is 10 mg, repeated up to three times daily.

The maximum recommended daily dose is 30 mg or 0.5 mg/kg body weight.

The maximum recommended treatment duration is 5 days.

Prevention of delayed chemotherapy induced nausea and vomiting (CINV) (Paediatric patients aged 9-18 years):

The recommended dose is 0.1 to 0.15 mg/kg body weight, repeated up to three times daily by oral route.

The maximum recommended dose in 24 hours is 0.5 mg/kg body weight.

Dosing table

Age	Body Weight	Dose	Frequency
9-18 years	30-60 kg	5 mg	Up to 3 times daily
15-18 years	Over 60 kg	10 mg	Up to 3 times daily

The maximum treatment duration is 5 days for prevention of delayed chemotherapy induced nausea and vomiting (CINV).

Frequency of administration:

A minimum interval of 6 hours between two administrations is to be respected, even if vomiting or rejection of the dose occurs.

Special population

Elderly:

In elderly patients a dose reduction should be considered, based on renal and hepatic function and overall frailty.

Renal impairment:

In patients with end stage renal disease (Creatinine clearance $\leq$ 15 ml/min), the daily dose should be reduced by 75%.

In patients with moderate to severe renal impairment (Creatinine clearance 15-60 ml/min), the dose should be reduced by 50%.

Hepatic impairment:

In patients with severe hepatic impairment, the dose should be reduced by 50%.

ROUTE OF ADMINISTRATION:

FOR ORAL USE ONLY

CONTRAINDICATIONS:

- Hypersensitivity to the active substance or to any of the excipients listed.
- Gastrointestinal haemorrhage, mechanical obstruction or gastrointestinal perforation for which the stimulation of gastrointestinal motility constitutes a risk.
- Confirmed or suspected pheochromocytoma, due to the risk of severe hypertension episodes.
- History of neuroleptic or metoclopramide-induced tardive dyskinesia.
- Epilepsy (increased crises frequency and intensity).
- Parkinson's disease.
- Combination with levodopa or dopaminergic agonists.
- Known history of methaemoglobinaemia with metoclopramide or of NADH cytochrome-b5 deficiency.
- Use in children less than 1 year of age.

WARNINGS AND PRECAUTIONS:**Neurological disorders**

Extrapyramidal disorders may occur, particularly in children and young adults, and/or when high doses are used. These reactions generally occur at the beginning of treatment, and can occur after a single dose. If extrapyramidal symptoms occur, metoclopramide should be discontinued immediately. These effects are generally completely reversible after treatment discontinuation; however, symptomatic treatment may be required (benzodiazepines in children, and/or anticholinergic antiparkinsonian medicinal products in adults).

An interval of at least six hours should be respected between each dose even if vomiting or rejection of the dose occurs, in order to avoid overdose.

Long-term treatment with metoclopramide may cause potentially irreversible tardive dyskinesia, particularly in elderly subjects. Treatment should not exceed 3 months because of the risk of tardive dyskinesia. Treatment must be discontinued if clinical signs of tardive dyskinesia occur.

Neuroleptic malignant syndrome has been described with metoclopramide in combination with neuroleptics and with metoclopramide monotherapy. Metoclopramide must be immediately discontinued if symptoms of neuroleptic malignant syndrome develop, and appropriate treatment should be initiated.

Particular caution should be exercised in patients with underlying neurological disorders, and in patients receiving other centrally-acting drugs.

Symptoms of Parkinson's disease may also be exacerbated by metoclopramide.

Methaemoglobinaemia

Methaemoglobinaemia, which could be related to NADH cytochrome-b5 reductase deficiency, has been reported. If this occurs, treatment must be immediately and permanently discontinued, and appropriate measures initiated (such as treatment with methylene blue).

Cardiac disorders

Serious cardiovascular undesirable effects, including cases of severe bradycardia, circulatory collapse, cardiac arrest and QT prolongation have been reported during administration of metoclopramide by injection, particularly via the intravenous route.

Particular caution should be exercised when administering metoclopramide, particularly via the intravenous route, in elderly subjects, patients with cardiac conduction disorders (including QT prolongation), patients with electrolyte imbalance, bradycardia, and patients taking other drugs known to prolong QT interval.

The intravenous injection must be given as a slow bolus (of at least 3 minutes' duration) in order to reduce the risk of undesirable effects (e.g. hypotension, akathisia).

Kidney or liver failure

In patients with kidney failure or severe liver failure, a dose reduction is recommended.

DRUG INTERACTIONS:

Metoclopramide and antiparkinsonian agents should not be prescribed with neuroleptics or with anticholinergic agents which suppress its digestive action.

Metoclopramide and antiparkinsonian agents should not be combined because of possible inhibition of their actions. Increased sedation with CNS depressants, impaired absorption of digoxin and Cimetidine and increased absorption from small intestine of Paracetamol, Tetracycline, levodopa may result with Metoclopramide use.

PREGNANCY AND LACTATION:

Although animal tests in several mammalian species have shown no teratogenic effects, treatment with Metoclopramide Tablets is not advised during the first trimester of pregnancy. Metoclopramide is excreted in breast milk and should not be given to the nursing mother.

SIDE EFFECTS:

Metoclopramide may cause extrapyramidal reactions, especially in young patients. Other adverse effects include bowel upsets, drowsiness and fatigue, dizziness, restlessness and anxiety. Galactorrhoea and gynaecomastia have been reported.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Extrapyramidal reactions may occur. In severe overdose the stomach should be emptied by aspiration and lavage. Severe extrapyramidal reactions should be treated with a slow I.V. injection of antiparkinsonian agents, such as benztropine mesylate, procyclidine HCl; or some antihistamine, such as diphenhydramine HCl. Diazepam may also be effective.

PACKING/PACK SIZE(S):

Blister pack of 10x10's and 100x10's.

**JAUHI UBAT DARIPADA KANAK-KANAK
KEEP OUT OF REACH OF CHILDREN**

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

DYNAPHARM (M) SDN. BHD. (65683-V)
2497, Mk. 1, Lorong Perusahaan Baru 5,
Kawasan Perusahaan Perai 3,
13600 Perai, Pulau Pinang, MALAYSIA

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